

Wisconsin Department of Health Services

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 0013774	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____	(X3) DATE SURVEY COMPLETED C 08/05/2024
NAME OF PROVIDER OR SUPPLIER AURORA RESIDENTIAL ALT INC 053		STREET ADDRESS, CITY, STATE, ZIP CODE 4525 RIVER DR PLOVER, WI 54467		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
M 000	INITIAL COMMENTS On 07/22/2024 with information gathered through 08/05/2024, Surveyor conducted a standard survey and 1 complaint investigation at Aurora Residential Alt Inc 053 in Plover. One (1) of 1 complaint will be substantiated. As a result of the survey, 4 deficiencies will be issued. Census: 4	M 000		
M 134	88.03(5)(e)1 SIGNIFICANT CHANGE TO THE RESIDENT Within 24 hours, a significant change in resident status, such as but not limited to an accident requiring hospitalization, missing from the home or a reportable death. A death shall be reported if there is reasonable cause to believe the death was related to use of a physical restraint or psychotropic medications, was a suicide or was accidental. This Rule is not met as evidenced by: Based on record review and interview, the licensee did not notify the department within 24 hours when 1 of 4 (Resident 1) residents had went missing from the home. Resident 1 was reported missing from the home 5 times since November 2023 until present. The licensee did not notify the department of the 5 incidents.	M 134		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

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M 134	Continued From page 1 Findings Include: On 07/11/2024, the department received a complaint regarding resident behaviors. On 07/22/2024, Surveyor conducted an onsite visit to the home. A sample of residents was selected, including the resident record of Resident 1. Resident 1 was admitted to the facility on 10/03/2017 with diagnoses to include autistic disorder, anxiety disorder, cognitive impairment, intermittent explosive disorder, mood disorder and articulation disorder. Resident 1's most recent ISP (Individualized Service Plan) dated 06/17/2024 stated Resident 1 requires 24 hour supervision with 30 minutes unsupervised allowed. Resident 1's ISP indicates Resident 1 has an elopement history but since moving to home Resident 1 has eloped much less often. "[Resident 1] somewhat often does leave the home with a need for space or activity but normally will return within 1 hour or staff have followed until [s/he] is ready to return. [Resident 1] does still struggle at time with controlling [his/her] impulse to bolt when [s/he] is upset. [S/he] has left the AFH [Adult Family Home] many times but almost always calms fairly quickly and/decides to return within the allotted 1 hour because [s/he] is aware there are natural consequences. [S/he] will occasionally tell staff [s/he] needs a walk but often times does not. [S/he] does usually give indicators that [s/he] is leaving but not always and may even try to wait for an opportunity to leave unnoticed. [S/he] is aware [s/he] has a max of 1 hour of unsupervised time and will usually return in that timeframe or stay on main road where staff is able to track [him/her] down and follow at a distance to give	M 134		

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M 134	Continued From page 2 [him/her] space. As of 2/21/18 [Resident 1] has a Project Lifesaver Band on [his/her] right ankle. The frequency number is [XXX.XXX] [Resident 1] has 1 hour of unsupervised time in which we hope [s/he] will decide to return to the site or staff. After that 1 hour OR if staff feels strongly that [his/her] intent is to elope and evade staff, staff should call the non emergency number. Tell the dispatcher that [s/he] is on the Project Lifesaver Program. This will initiate actions on their part of calling in officers who are trained on Project Lifesaver and have the equipment needed. They will need to know [his/her] frequency number [XXX.XXX], last known location, and what [s/he] is wearing. It is important that when two staff are present that [Resident 1] is followed immediately, as the Project Lifesaver has had problems finding [him/her] when there is too much distance from the receiver. [Resident 1] has been provided with a walkie talkie so that staff can check on [him/her] while [s/he] is unsupervised. [S/he] also has a vest to wear for safety reasons with reflective stripe. [S/he] also has a sign out sheet available that encourages [him/her] to tell staff as well as reminding [him/her] of the safety measures that are in place for [his/her] protection. 6/17/24 UPDATE: Everyone agrees that [Resident 1's] unsupervised time should be reduced to 30 minutes." Additionally, Resident 1's BSP (Behavior Support Plan) states, "6/21 [Resident 1's] guardian, [MCO] team and Aurora staff and Program Director all agreed to increase [Resident 1's] unsupervised to 1 hour. *6/17/24 [Resident 1's] guardian, [MCO] team, Aurora staff and Program Director all agree to decrease [Resident 1's] unsupervised to 30 minutes. Behavior 1 - Elopement-leaving the supervision of staff without consent. If [Resident 1] should leave the home or supervision while in the community and 2 staff are present, one staff are to follow [him/her] or	M 134		

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M 134	Continued From page 3 attempt to locate [him/her] immediately. If you are following [Resident 1], give [him/her] some space and walk with [him/her]. Often walking and exerting some energy when [s/he] is upset or anxious will help calm [him/her]. [S/he] will often return willing with staff once [s/he] has calmed. Remember [Resident 1] has a history of hiding in small space around a home. Be sure to search closets, cupboards and the garage if you suddenly can not find [him/her]. If [Resident 1] leaves the site for a walk or run [s/he] will be allowed 30 minutes to return before law enforcement is contacted. Contact the Call Center to inform them of the situation. If you are unable to locate [Resident 1] or [s/he] has not returned within 30 minutes call the Call Center for further support and direction from management. Notify Law Enforcement if you are unable to locate [Resident 1] within 30 minutes and after searching the immediate area. Let them know that [Resident 1] is a vulnerable adult placed in an Aurora Adult Family home and that [s/he] wears a Project Lifesaver Band." On 07/23/2024, Surveyor reviewed Resident 1's incident reports. The following was noted: *On 11/02/2023, Resident 1 was noted to be physically aggressive towards staff, knocking the staff member to the floor. Staff member called 911 and Resident 1 walked out the front door. The police picked Resident 1 up a couple of blocks from the site and brought Resident 1 back. The incident report indicates DQA (Division of Quality Assurance) was notified. *On 11/24/2023 at 3:30PM, Resident 1 told staff s/he was going to go for a walk, staff reminded Resident 1 to be back home in 30 minutes. At 4pm, Resident 1 had not arrived. Staff checked outside to see if Resident 1 was on way back, but	M 134		

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M 134	Continued From page 4 did not see Resident 1. Staff waited another 5 minutes and no sign of Resident 1 returning. Staff called Project Lifesaver. Police located Resident 1 down the street and brought him/her back at 4:32PM. The incident report indicates DQA was notified. *On 04/15/2024 at 3:15PM Resident 1 left out the backdoor without telling staff where s/he was going or what s/he was doing, s/he also did not bring a walkie talkie with him/her. At 4:00PM, after not being able to locate Resident 1, staff called the police. At 4:10PM staff found Resident 1 outside leaning on the front railing near the front door. Resident 1 stated s/he went for a walk. The incident report indicates DQA was not notified of the incident. *On 05/20/2024 at 4:45PM staff were making dinner and Resident 1 went out the front door. After 20 minutes, staff looked around the house and down the street for Resident 1 and did not see him/her. Staff called program director to confirm 60 minute window before calling police per ISP. After 60 minutes without Resident 1 returning, staff called police to locate Resident 1. Police brought Resident 1 home after finding him/her walking on a trail off of Grant Ave. near Highway 54. Resident 1 had indicated s/he crossed Highway 54 going to from girlfriends house. The incident report indicates DQA was notified of this incident. *On 06/18/2024 at approximately 3:05PM Resident 1 went outside and then ran off. Staff went to follow him/her but s/he was out of sight. Another staff member went looking for him/her in the van. This staff member eventually found Resident 1 by the highway around 3:40PM and asked him/her to get in van and Resident 1 said no and took off again into the woods. Staff member called Project Lifesaver for his/her safety as they were unable to locate him/her and it had	M 134		

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M 134	Continued From page 5 been outside of his/her 30 minute time frame. Police located Resident 1 and returned him/her home. The incident report indicates DQA was notified of this incident. On 07/23/2024, Surveyor reviewed the department's facility folder and was unable to locate any incident reports the facility reported regarding Resident 1. On 07/31/2024 at approximately 10:00AM, Surveyor interviewed Program Director B. Program Director B indicated s/he was unsure if the reports were sent to the department. Program Director B stated Licensee A would be responsible for sending the reports to the department. On 08/05/2024 at 9:00AM, Surveyor interviewed Licensee A. Licensee A verified the reports noted above were not submitted to the department. Licensee A stated there was turnover in management and a misunderstanding on what reports to submit to the department. Licensee A stated s/he was unaware of the incident reports mentioned above and instructed Program Director B not to mark on the incident report that the department was made aware of the report until they have discussed that the report has been submitted to the department. Cross reference: M 0230 DHS 88.04(2)(f) Condition of Risk	M 134		
M 230	88.04(2)(f) CONDITION WHICH REPRESENTS RISK OR HARM The licensee may not permit the existence or continuation of a	M 230		

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M 230	Continued From page 6 condition in the home which places the health, safety or welfare of a resident at substantial risk of harm. This Rule is not met as evidenced by: Based on record review and interview, the provider permitted the existence and continuation of a condition of substantial risk and harm to occur at the facility. Resident 1's resident record noted several instances of sexually inappropriately touching Resident 2. Licensee A did not address the initial reports, permitting the existence and continuation of a condition of substantial risk and harm to occur at the facility. Findings include: The facility is licensed to serve up to 4 residents who may have one or more of the following conditions: Traumatic Brain Injury, physically disabled, and/or developmentally disabled (DD). On 07/11/2024, the department received a complaint alleging a resident had been sexually inappropriately touching other residents over several months. On 07/22/2024, Surveyor conducted an onsite visit to the facility. Resident 1 was at adult day center during Surveyor's visit. Upon entrance, Resident 2 was in the shower and then came out to living room area and sat in recliner chair. During visit, Surveyor observed Resident 2 go for a walk outside with caregiver, eat lunch and watch a movie with roommate in the living room. On 07/23/2024 Surveyor reviewed Resident 1	M 230			

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M 230	Continued From page 7 and 2's resident records. The following was noted. Resident 1 Resident 1 was admitted to the facility on 10/03/2017 with diagnoses to include autistic disorder, anxiety disorder, cognitive impairment, intermittent explosive disorder, mood disorder and articulation disorder. Resident 1's most recent ISP (Individualized Service Plan) dated 06/17/2024 stated Resident 1 requires 24 hour supervision with 30 minutes unsupervised allowed. Additionally, Resident 1's ISP indicated Resident 1 is "at risk to abuse other vulnerable adults...sexually inappropriate behavior. Responds to redirection: - Note: [Resident 1] seems to crave physical contact and will often seek this out from staff. It is ok to give [him/her] a hand shake or verbal praise, but staff needs to be very careful because [Resident 1] could misinterpret what any physical contact means. If [Resident 1] hugs, invades personal space, or touches anyone without permission staff must redirect [him/her] and ask for personal space, as well as let [him/her] know that is not appropriate." On 07/23/2024 through 07/29/2024, Surveyor reviewed Resident 1's incident reports and resident daily notes. The following was noted: *A resident profile note in the Rtask system titled "Hiding/masturbation Behavior" dated 05/24/2023 stated, "At dinner time, [Resident 1] did not reply to staff calling for [him/her] including calling down to the basement. Staff then began looking for [him/her] inside and outside the house and then went down to the basement while still asking out if [s/he] was there. Upon turning the lights on, [Resident 1] sat up on one of the couches fully nude. Staff asked if [s/he] was ok, and why didn't	M 230		

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M 230	Continued From page 8 [s/he] reply. [Resident 1] replied that [s/he] did and was just "working out." *A resident profile note dated 9/9/23 stated, "I noticed [Resident 1] laying with [his/her] head on [Resident 2's] lap. Since [Resident 2] does not have the ability to verbalize what [s/he] is feeling or experiencing it is imperative that we document this behavior or any other physical contact beyond expected friendship contact is seen." *A resident profile note dated 9/24/23 stated, "Incident follow-up Upon checking the tablet for sites that we wouldn't want [his/her] housemate to visit, this writer discovered two visits to pornhub.com and xnxx.com dated September 21, 2023 beginning at 8:56 PM." *On 01/15/2024 a Service Recap note states, "Resident 1 was sitting on the couch in the sun room and there was a staff and another housemate in the sun room as well. The staff in the room walked out and this staff was right around the corner in the living room, after the other staff walked out this staff heard [Resident 1] say, "come here. come sit by me." to the other housemate in the room with [him/her]. This staff knows that [Resident 1] this [sic] not suppose to be sitting in close proximity to any of the other housemates, especially when there isn't a staff directly supervising [him/her]. This staff moved from the living room into the sun room in order to keep an eye on things and noticed upon entering the room that [Resident 1] had [his/her] on the housemates upper leg. When this staff noticed this they prompted the other housemate to go and sit somewhere else, and the other staff on shift asked [Resident 1] if there was a reason they were sitting so closely with all the other seating available in the house, and [Resident 1]	M 230		

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M 230	Continued From page 9 responded "no". Please note that [Resident 1] has touched other house mates inappropriately in the past, is aware that [s/he] is not to be in that close of proximity to others, and to be vigilant and aware of this behavior moving forward, especially with the changes happening in [his/her] love live recently." *On 01/18/2024 an incident report stated, "Resident 1 was sitting on the couch in the sun room and there was a staff and another housemate in the sun room as well. The staff in the room walked out and this staff was right around the corner into the living room when they heard [Resident 1] say "come here. come sit by me." to the other housemate in the room with [him/her]. Staff entered the sun room in order to keep an eye on things and noticed upon entering the room that [Resident 1] had [his/her] on [his/her] housemates upper leg. When writer entered the room, writer prompted the other housemate to go and sit somewhere else and let [Resident 1] know that there is plenty of seating and reminded [him/her] that it is important to give personal space." *A resident profile note dated 2/19/24 indicated Resident 1 was making statements about having sex with another day center consumer. A statement from the note stated, "We are at this time unsure if this is true, but this is concerning and needs to be known by all parties involved." *A resident profile note dated 4/29/24 indicated day center called the house to speak with program manager about Resident 1's recent increase over the weekend of phone harassments of 3 fe/males that s/he attends day center with.	M 230			

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M 230	Continued From page 10 *A resident profile note dated 5/21/24 stated, "The consumer was discovered going into another consumers bedroom on the opposite side of the house in the middle of the night. It has been a recent occurrence that this consumer will avoid detection by going through [his/her] shared bathroom and housemates bedroom, thereby avoiding [his/her] alarm from going off so as to remain undetected. It is imperative that all staff keep this consumer under constant watch for the safety of the consumer as well as the other consumers in the house." *On 07/05/2024 an incident report stated, "At around 8pm on 6/27/24 [Resident 1] was sitting at the table eating [his/her] snack with [his/her] housemates when [his/her] housemates bit [him/her] on the side of the head with an open hand, causing [him] to fall off [his/her] chair and onto the ground. [Resident 1] was able to get up on [his/her] own without staff assistance and did not have any visible injuries. Staff examined [Resident 1] to see if [s/he] had an injuries, there were none visible and [s/he] did not complaint of any pain." The incident report indicates management found a review of the events as accurate and the report was completed by Caregiver I and reviewed by Program Director B. *On 07/25/2024, Surveyor received a picture from Caregiver I that was of the original incident report submitted by Caregiver I regarding the 06/27/2024 incident between Resident 1 and Resident 3. It states the following, "On 06/27/2024 at 7:45pm, [Resident 3], [Resident 1] and [Resident 4] were at the table eating their 8pm snack which consisted of a small bowl of Cheerios with a glass of water. As they were eating, this staff went to put the bag of Cheerios back in the pantry, then walk to the other side of	M 230		

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M 230	Continued From page 11 the counter to observe them eating. As this staff's back was turned, [Resident 1] started yelling while stuff was being tossed around the dining area. This staff turned and saw [Resident 3] throwing everyone's water cups and bowls of Cheerios all over the table and floor. [Resident 4] immediately got up and ran away to [Resident 4's] chair in the living room. Due to [Resident 1] being the closest and loudest person in this situation, [Resident 3] open-handedly hit [Resident 1] across the top of the head causing [Resident 1] to fall on the floor. As [Resident 1] was on the floor, [Resident 4] quickly shot up and charged towards this staff. As [Resident 4] was running towards this staff, this staff loudly and sternly stated, "Absolutely not, that is not okay; go to your room right now!" then full force, open-handedly, hit this staff on the right side of their face. After [Resident 3] hit this staff, [Resident 3] stormed into [Resident 3's] room and slammed the door shut. This staff immediately ran to [Resident 1] to make sure [Resident 1] was okay. [Resident 1] showed this staff where [Resident 3] hit [him/her], then stormed out the front door from frustration. A few minutes later, [Resident 1] came back in the house through the back door. After this staff checked their face for any immediate, noticeable injury, this staff verified with [Resident 1] if [Resident 1] was hurt or needed to be seen by a doctor, then proceeded to start the clean up process of the mess in the dining area... This staff has been employed here for only two weeks and was alone the night of the incident." *On 07/05/2024 an incident report stated, "Staff just finished cleaning dinner around 5:30pm and entered the living room to find [Resident 1] sitting next to [his/her] housemate on the couch. At this time [Resident 1] was holding housemates hands in between [his/her] own legs near [his/her]	M 230		

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M 230	Continued From page 12 genitals (over [his/her] pants). Staff quickly redirected the housemate to another location of the house. *On 07/18/2024 an incident report stated, " On 7/18/2024 around 7pm, this staff went downstairs to question [Resident 1] if [s/he] had taken another consumer's tablet. This staff asked [Resident 1] to come out of the room [s/he] was in, but [s/he] couldn't open the door and was locked inside. This staff attempted multiple times until after 8pm to get [Resident 1] to open the door, but found [s/he] was legitimately locked inside the room. After working together to attempt to open the door, this staff was unsuccessful. This house's PD[Program Director], [Resident 1] admitted to this staff that [s/he] had taken the other consumer's tablet. This staff then went onto the tablet's search history and found pornography searches with timestamps from within the hour [Resident 1] was in that locked room. Resident 2 Resident 2 was admitted to the facility on 05/21/2012 with diagnoses to include autism, global assessment of functioning, mental retardation and seizures. Resident 2 is under court appointed guardianship. Resident 2's most recent ISP dated 04/02/2024 indicated Resident 2 "will say some words. Needs to be encouraged to speak. Often will parrot what is said to [him/her]. [Resident 2] will use ASL [American Sign Language] and other means to communicate his wants and needs. Its important that staff learn [his/her] routine and anticipate what [s/he] needs to assist with their understanding of what [s/he] is wanting...Client is at risk to be abused - specify measures to minimize risk: -Note: Due to diagnosis, cognitive ability as well as communication challenges [Resident 2] has 24/7	M 230			

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M 230	Continued From page 13 supervision for [his/her] safety as well as the safety of others. See BSP for specifics...Sexually Inappropriate - masturbation in non-private areas, fixating on another person then wanting to sit by them and rub [his/her] groin, attempt to touch other or get on top of them. Attempt to follow others into the bathroom or a bedroom. [Resident 2] will need direct line of sight in the home if [s/he] indicates an attraction to any of [his/her] peers. If [Resident 2] begins to rub [his/her] private area in the general living area or other inappropriate places, direct [him/her] to [his/her] room for privacy. [S/he] is usually very redirectable with this behavior. Watch for [Resident 2] to have a particular interest in a peer or attempt to approach or touch a peer. [His/her] attraction for a peer is usually apparent and [s/he] is easily redirected away from the person. Remember [Resident 2] is an opportunist if is[sic] has a sexual interest in someone. [S/he] will take advantage of times when staff are distracted doing something else to attempt to approach the person. It is important for staff to look for signs that [Resident 2] may be attracted to someone and be vigilant in supervising and redirecting [Resident 2] if this occurs." On 07/23/2024 through 07/29/2024, Surveyor reviewed Resident 2's incident reports and resident daily notes. The following was noted: *On 07/10/2024 an incident report stated Resident 2 was "involved in inappropriate touching by fellow Consumer...After dinner clean up, about 5:30 pm, this staff entered the living room to find [Resident 2] sitting on the couch next to [his/her] housemate, [his/her] housemate was holding [Resident 2's] hands near [his/her] genitals (over [his/her] pants). Staff quickly redirected [Resident 2] to a different area of the	M 230		

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M 230	Continued From page 14 house, there were no issues for the remainder of the shift." Interviews On 07/22/2024 at 9:40AM, Surveyor interviewed Program Manager C. Program Manager C stated s/he has been in this role at the home for about a year. Program Manager C indicated Resident 1 was at adult day center, which s/he attends 5 days a week. Program Manager C stated s/he has concerns that Resident 1 is being sexually inappropriate with other residents and management is not doing anything about it. Program Manager C stated Resident 1 will steal other residents tablets and look up pornography websites, Resident 1 has been caught going in and out of other resident rooms, Resident 1 was seen by staff with his/her hands on another resident's upper thigh, Resident 1 followed a female staff member to the bathroom while on an outing and most recently Resident 1 was seen by staff sitting on couch next to Resident 2 with Resident 2's hands between Resident 1's legs near genitals. Program Manager C stated s/he also witnessed Resident 1 put his/her face in Resident 2's lap and after Program Manager C separated the residents, Program Manager C noted Resident 2 to be sexually aroused. Program Manager C stated s/he asked Resident 1 what s/he was doing and Resident 1 stated Resident 2 was my pillow. Program Manager C stated s/he documented this incident but can't find the incident report or documentation in Resident 1's record now. Program Manager C stated day services had concerns as well regarding inappropriate touching including kissing and spooning of clients at adult day center. Program Manager C stated Resident 1 knows what s/he is doing is wrong and will find times to be sexually inappropriate when staff are not	M 230			

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M 230	Continued From page 15 around. However, Resident 2 has limited speech and cognitively is at a 3 year old level and would not understand what is happening and would not speak up for themselves. Program Manager C stated Resident 1 has a history of sexually inappropriate behaviors going back a few years when Resident 1 followed a girl in the community on numerous occasions and the police had to get involved. Additionally, Resident 1 has used the house phone to call friends continuously, scarring the friends, leading the friends to block the house number. Program Manager C some of the incidents are documented and others are not as staff were unsure if they needed to document them. Program Manager C stated Resident 1's MCO (Managed Care Organization) was just made aware of the sexual behaviors last week and Resident 1's guardian was never notified of the inappropriate touching. On 07/22/2024 at 10:15AM, Surveyor interviewed Caregiver E. Caregiver E shared s/he has been working at the home since January 2024 on the day shift. Caregiver E stated Resident 1 is usually at day center during his/her shift so s/he doesn't have much interaction with Resident 1 because by the time Resident 1 comes home from day center, Caregiver E is done with shift. Caregiver E stated s/he has not witnessed any sexual behaviors by Resident 2, however has heard of incidents. Caregiver E stated s/he heard of an incident with Resident 1 and Resident 2 that happened about 2 weeks ago. Caregiver E stated Resident 1 and 2 were sitting on couch together and Resident 1 was holding Resident 2's hands in between Resident 2's legs near the genitals. Caregiver E stated s/he also heard of an incident where Resident 1 was found with his/her head on Resident 2's lap. Caregiver E also indicated staff have said they have seen Resident 1 "running	M 230		

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M 230	Continued From page 16 out" of Resident 2's room during the middle of the night. Caregiver E stated this wasn't documented in any resident's record. Caregiver E stated Resident 1 knows what s/he is doing is wrong and that after the incidents, Resident 1 is remorseful. Caregiver E stated s/he is aware of an incident last summer with Resident 1 and a person in the community. Resident 1 followed this person on numerous occasions throughout the community and the police department needed to get involved. More recently, Caregiver E stated Resident 1 eloped from the home and walked to another home where 2 friends from day center live. The 2 friends were afraid to go to the day center the next day due to the visit and numerous calls Resident 1 made to their home. Caregiver E also stated the other day Resident 1 stole another resident's tablet and was found to be watching disturbing strangulation pornography on it. And a few days after that, Resident 1 was seen watching a movie on the TV in the living room that would pause on a half naked person every 10 seconds. Caregiver E stated Resident 1 also has a "fascination" with Caregiver F. During an outing to the zoo, Resident 1 followed Caregiver F to the bathroom and tried to peek in the windows, even after Resident 1 was told it was the opposite sex bathroom. Caregiver E stated Resident 1 will stare and peer down shirts of the opposite sex. Caregiver E stated staff are uncomfortable around Resident 1. On 07/23/2024 at 11:00AM, Surveyor interviewed Caregiver D. Caregiver D stated s/he has been employed at the home for about 5 years and works the 2:00pm-10:00pm shift. Caregiver D indicated s/he was familiar with Resident 1. Caregiver D acknowledged Resident 1 has sexual behaviors towards other residents and has had an increase in sexual expressions recently.	M 230		

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M 230	Continued From page 17 Caregiver D stated Resident 2 has always been "touchy feely" but more "perverted" in the last 2 years. Caregiver D recalled an incident when Resident 1 put his/her face in Resident 2's private area while Resident 2 was sitting on the couch. Caregiver D intervened and Resident 1 got up and swore at Caregiver D. Another incident Caregiver D recalled was when Resident 1 used Resident 2's hands to touch Resident 1's private area. When staff approached, Resident 1 scooted to another chair in living room. Caregiver D indicated a previous employee witnessed Resident 1 in bed with Resident 2 in Resident 2's room. Caregiver D stated the previous employee did not document the incident and is no longer employed at the facility. Caregiver D stated other employees have wrote incident reports and can not locate the report anymore or the reports have been changed by management. Caregiver D stated the incident reports filled out by staff "are much different than what staff are writing." Caregiver D stated some employees do not feel comfortable working alone with Resident 1, as Resident 1 will follow them around the home "looking for a reason to touch or stare at the staff member." Caregiver D indicated the previous program manager would let Resident 1 lay by him/her on the couch with Resident 1's head in program manager's lap. After program manager left, that is when caregivers started to notice an increase in Resident 1's sexual behaviors. Caregiver D stated Resident 1's sexual behaviors "so common that can catch [Resident 1] before it happens, that's why they are not documented. [Resident 1] shouldn't be with 3 disabled [residents]." Caregiver D stated recently Resident 3 had a home visit and Resident 3's family members asked what was going on in the house. Resident 3 said Resident 1 "molested" him/her in his/her bedroom. Resident 3's family members	M 230		

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M 230	Continued From page 18 called the police. On 07/25/2024 at 11:00AM, Surveyor interviewed Caregiver I. Caregiver I stated s/he has been working at the home just over a month and works the 2-10pm shift. Caregiver I stated s/he hasn't witnessed any inappropriate sexual touching by Resident 1, however, have seen/heard of previous reports. Caregiver I stated s/he was aware of an incident where Resident 1 had his/her head in the "crotch" of Resident 2 and an incident where Resident 1 followed a caregiver of the opposite sex to the bathroom while on an outing. Caregiver I stated caregivers are instructed to document behaviors on incident report or in behavior log. The reports and log are sent to Program Director B for review. Program Director B will revise them and then officially submit them to all parties. Caregiver I indicated s/he has reported 1 behavior and 1 incident report since starting at the home. Caregiver I stated the incident report was involving a physical altercation between Resident 3 and Resident 1. During dinner time, Resident 3 hit Resident 1 across the head, knocking Resident 1 off his/her chair. Caregiver I stated s/he filled out an incident report with 9 lines of text and when Caregiver I went back to check on the incident report, it was modified and only 3 or 4 lines of text. Caregiver I stated s/he has pictures of what Caregiver I originally submitted. [See Exhibit 1] Caregiver I stated the day of the incident s/he was working alone, the other caregiver had left early or didn't show up. Caregiver I stated this was the first time she had worked alone while employed at the home. Caregiver I stated the behavior report s/he submitted was regarding a time when Resident 1 stole another residents tablet and was found in basement looking at pornography sites on the tablet. Caregiver I stated again, s/he was the only	M 230		

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M 230	Continued From page 19 caregiver in the home at the time due to the other caregiver leaving early. Caregiver I stated s/he is uncomfortable being in the home by self with Resident 1 due to Resident 1's sexual inappropriateness. Caregiver I had indicated s/he had conversations with the police department regarding Resident 1 and the sexual behaviors. On 07/24/2024 at 8:45AM, Surveyor interviewed Detective J. Detective J had verified an ongoing police investigation into Resident 1's sexual assault allegations and the falsifying of documentation by Program Director B. At the time of writing, the police department's investigation had not concluded. On 07/31/2024 at approximately 10:00AM, Surveyor interviewed Program Director B. Program Director B stated s/he has been in this position just under a year. As Program Director, Program Director B stated s/he helps the program manager with household duties, updates care plans of residents as needed and facilitates monthly team meetings with staff at the home. Program Director B indicated the home runs 3 shifts, with 2 staff working the AM (6a-2p), 2 staff working the PM (2p-10p) and 1 staff working Nights (10p-6a). Program Director B stated in the past they have been short staffed due to staffing crisis or staff calling in sick then single staffed but recently there has been consistently 2-3 staff per shift for AM and PM shifts. Program Director B stated s/he has heard many different things from different people regarding Resident 1. Staff have said they don't like him/her, referring to Resident 1 as a "creep" or "pedophile." Program Director B stated there are other residents in the home with sexual behaviors as well. Program Director B acknowledged that lack of supervision can create an opportunity for Resident 1 to do something	M 230		

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M 230	Continued From page 20 s/he shouldn't be doing. Program Director B stated staff have discussed their concerns at the monthly team meetings. Program Director B stated that some of the things staff are reporting were not reported to Program Director B right away. Program Director B stated s/he has since told staff they need to be documenting the behaviors right away. Program Director B stated that staff are instructed to fill out an incident report for physical altercation, elopement, injury, sexual abuse, etc. Program Director B stated s/he does review the incident reports and will correct grammar, spelling and take out information if it is not factual or not related to the incident. Program Director B stated s/he was trained by management to do it this way, to keep the report factual. Program Director B acknowledged the difference in the original 06/27/2024 incident report submitted by Caregiver I and the revised report located in Resident 1's record. Program Director B stated s/he did review the incident with Caregiver I. Based on that interview, Program Director B stated s/he felt there was information contained in the original report that was not pertinent to the incident so deleted it. Program Director B acknowledged s/he did not mean to do anything malicious and was not hiding any information, just trying to keep the report clear and concise and pertinent to the incident. Additionally, Program Director B stated s/he was unaware of any reports of Resident 1 placing his/her head in the lap of Resident 2. Program Director B stated that incident might have happened with previous program director. Program Director B stated s/he did not mean for the reports to come across as "downplayed." As for Resident 3's most recent statements about Resident 1 touching his/her genital area, Program Director B stated s/he became aware of those allegations the day	M 230		

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M 230	Continued From page 21 before Resident 1 was discharged from the facility. Program Director B was not aware of any current investigation the home was doing on those allegations since Resident 1 had been discharged already. Program Director B acknowledged the need for staff to receive training on incident reports, sexuality/healthy relationships, documenting, reporting and investigating abuse allegations. On 08/05/2024 at 9:00AM, Surveyor interviewed Licensee A. Licensee A acknowledged Resident 1 had sexual expressions towards other residents in the home over the last several months. Licensee A stated there had been some change over in management and several of the incidents were not reported to Licensee A. Licensee A inquired about the falsifying of incident reports as Program Director B had mentioned there was some accusations against Program Director B. Licensee A stated directors do have the ability to edit for grammar and to clarify any details for the report to make sense but should not alter details of the report that would affect the ability of staff to understand what happened and help them to prevent something from happening in the future. Licensee A indicated understanding of this and stated, "there was a failure of supervision on our part." Cross Reference: M 0134 DHS 88.03(5)(e)1 Significant Change to the Resident M 0424 DHS 88.06(3)(f) Review of ISP M 0450 DHS 88.07(2)(b)6 Notification of Changes	M 230			
M 424	88.06(3)(f) REVIEW OF ISP	M 424			

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M 424	Continued From page 22 The individual service plan shall be reviewed at least once every 6 months by the licensee, the resident or the resident's guardian, the service coordinator, if any, the designated representative, if any, the placing agency with the resident participating in a manner appropriate for the resident's level of understanding and method of communication. This review is to determine continued appropriateness of the plan and to update the plan as necessary. A plan shall be updated, in writing, whenever the resident's needs or preferences change substantially or when requested by the resident or the resident's guardian. This Rule is not met as evidenced by: Based on record review and interview, the provider did not update 1 of 4 (Resident 1) resident's care plan when the resident's needs or preferences change substantially. Resident 1 was noted to have an increase in sexual behaviors. Resident 1's care plan was not updated to reflect the increase in sexual behaviors. Additionally, Resident 1's care plan did not reflect a change in phone usage, limiting Resident 1's usage to the hours of 6pm-8pm daily. Findings Include: On 07/11/2024, the department received a complaint regarding resident behaviors.	M 424			

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M 424	Continued From page 23 On 07/22/2024, Surveyor conducted an onsite visit to the facility. A sample of residents was selected including the record of Resident 1. Resident 1 was admitted to the facility on 10/03/2017 with diagnoses to include autistic disorder, anxiety disorder, cognitive impairment, intermittent explosive disorder, mood disorder and articulation disorder. Resident 1's most recent ISP (Individualized Service Plan) dated 06/17/2024 stated Resident 1 requires 24 hour supervision with 30 minutes unsupervised allowed. Behaviors Resident 1's ISP indicates Resident 1 is "at risk to abuse other vulnerable adults...sexually inappropriate behavior. Responds to redirection: - Note: [Resident 1] seems to crave physical contact and will often seek this out from staff. It is ok to give [him/her] a hand shake or verbal praise, but staff needs to be very careful because [Resident 1] could misinterpret what any physical contact means. If [Resident 1] hugs, invades personal space, or touches anyone without permission staff must redirect [him/her] and ask for personal space, as well as let [him/her] know that is not appropriate." On 07/23/2024 through 07/29/2024, Surveyor reviewed Resident 1's incident reports and resident daily notes. The following was noted: *A resident profile note in the Rtask system titled "Hiding/masturbation Behavior" dated 05/24/2023 stated, "At dinner time, [Resident 1] did not reply to staff calling for [him/her] including calling down to the basement. Staff then began looking for [him/her] inside and outside the house and then went down to the basement while still asking out if [s/he] was there. Upon turning the lights on,	M 424		

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M 424	Continued From page 24 [Resident 1] sat up on one of the couches fully nude. Staff asked if [s/he] was ok, and why didn't [s/he] reply. [Resident 1] replied that [s/he] did and was just "working out." *A resident profile note dated 9/9/23 stated, "I noticed [Resident 1] laying with [his/her] head on [Resident 2's] lap. Since [Resident 2] does not have the ability to verbalize what [s/he] is feeling or experiencing it is imperative that we document this behavior or any other physical contact beyond expected friendship contact is seen." *A resident profile note dated 9/24/23 stated, "Incident follow-up Upon checking the tablet for sites that we wouldn't want [his/her] housemate to visit, this writer discovered two visits to pornhub.com and xnxx.com dated September 21, 2023 beginning at 8:56 PM." *On 01/15/2024 a Service Recap note states, "[Resident 1] was sitting on the couch in the sun room and there was a staff and another housemate in the sun room as well. The staff in the room walked out and this staff was right around the corner in the living room, after the other staff walked out this staff heard [Resident 1] say, 'come here. come sit by me,' to the other housemate in the room with [him/her]. This staff knows that [Resident 1] this [sic] not suppose to be sitting in close proximity to any of the other housemates, especially when there isn't a staff directly supervising [him/her]. This staff moved from the living room into the sun room in order to keep an eye on things and noticed upon entering the room that [Resident 1] had [his/her] on the housemates upper leg. When this staff noticed this they prompted the other housemate to go and sit somewhere else, and the other staff on shift asked [Resident 1] if there was a reason	M 424		

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M 424	Continued From page 25 they were sitting so closely with all the other seating available in the house, and [Resident 1] responded 'no.' Please note that [Resident 1] has touched other house mates inappropriately in the past, is aware that [s/he] is not to be in that close of proximity to others, and to be vigilant and aware of this behavior moving forward, especially with the changes happening in [his/her] love life recently." *On 01/18/2024 an incident report stated, "Resident 1 was sitting on the couch in the sun room and there was a staff and another housemate in the sun room as well. The staff in the room walked out and this staff was right around the corner into the living room when they heard [Resident 1] say 'come here. come sit by me.' to the other housemate in the room with [him/her]. Staff entered the sun room in order to keep an eye on things and noticed upon entering the room that [Resident 1] had [his/her] on [his/her] housemates upper leg. When writer entered the room, writer prompted the other housemate to go and sit somewhere else and let [Resident 1] know that there is plenty of seating and reminded [him/her] that it is important to give personal space." *A resident profile note dated 2/19/24 indicated Resident 1 was making statements about having sex with another day center consumer. A statement from the note stated, "We are at this time unsure if this is true, but this is concerning and needs to be known by all parties involved." *A resident profile note dated 4/29/24 indicated day center called the house to speak with program manager about Resident 1's recent increase over the weekend of phone harassments of 3 fe/males that s/he attends day	M 424		

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M 424	Continued From page 26 center with. *A resident profile note dated 5/21/24 stated, "The consumer was discovered going into another consumers bedroom on the opposite side of the house in the middle of the night. It has been a recent occurrence that this consumer will avoid detection by going through [his/her] shared bathroom and housemates bedroom, thereby avoiding [his/her] alarm from going off so as to remain undetected. It is imperative that all staff keep this consumer under constant watch for the safety of the consumer as well as the other consumers in the house." *On 07/05/2024 an incident report stated, "Staff just finished cleaning dinner around 5:30pm and entered the living room to find [Resident 1] sitting next to [his/her] housemate on the couch. At this time [Resident 1] was holding housemate's hands in between [his/her] own legs near [his/her] genitals (over [his/her] pants). Staff quickly redirected the housemate to another location of the house." *On 07/18/2024 an incident report stated, "On 7/18/2024 around 7pm, this staff went downstairs to question [Resident 1] if [s/he] had taken another consumer's tablet. This staff asked [Resident 1] to come out of the room [s/he] was in, but [s/he] couldn't open the door and was locked inside. This staff attempted multiple times until after 8pm to get [Resident 1] to open the door, but found [s/he] was legitimately locked inside the room. After working together to attempt to open the door, this staff was unsuccessful. This house's PD[Program Director], [Resident 1] admitted to this staff that [s/he] had taken the other consumer's tablet. This staff then went onto the tablet's search history and found pornography	M 424		

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M 424	Continued From page 27 searches with timestamps from within the hour [Resident 1] was in that locked room." Phone Usage Resident 1's ISP dated 06/17/2024 indicates, "[Resident 1] is able to use telephone to call in an emergency - Note: [Resident 1] is able to dial and use a phone but cannot be relied on to always understand when to do this." Resident 1's ISP did not indicate any interventions related to the use of the phone. On 07/23/2024 through 07/29/2024, Surveyor reviewed Resident 1's incident reports and behavior notes. The following was noted: *On 01/09/2024 at service recap note stated, "[Resident 1] is only allowed to use the phone between 6pm and 8pm due to harassment complaint from multiple other outside of Aurora. At around 330pm, [Resident 1] grabbed the phone and proceeded to call [his/her] friend. This staff reminded [him/her] of the guidelines and [s/he] just ignored me and stayed on the phone for another 15 minutes. PM [program manager] called to talk to [him/her] about it and [Resident 1] refused to take the phone. When this staff told [him/her] who was on the phone, [Resident 1] replied, "I don't care!" And then mumbled some obscenities. [Resident 1] also refused to take out the garbage and recycling this evening." *On 03/26/2024 a service recap note stated, "...Due to past behaviors of harassing others with the phone, [s/he] is only allowed to use the phone between the hours of 6pm and 8pm. And, [s/he] must be monitored by staff while talking..." *On 04/29/2024 a resident profile note stated, "[Adult day center] called the house to speak with	M 424			

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M 424	Continued From page 28 me concerning [Resident 1's] recent increase over the weekend of phone harassments of three friends that [s/he] attends [Adult Day Center] with. [S/he] was wondering if anything is going on at the house that could explain why the sudden increase in calls, and wanting a [boy/girl]friend. I explained to [him/her] that nothing has really changed beyond the seasonal changes and that I would send a note about reminding staff that if [Resident 1] is to use the phone it should only occur between 6-8:00PM and has to be in common areas so staff can monitor to be sure [s/he] is not calling the same people excessively..." Interviews On 07/23/2024 at 9:40AM, Surveyor interviewed Caregiver F. Caregiver F stated s/he has been working at the home since April 2024 and works the 2-9pm shift. Caregiver F indicated s/he was familiar with Resident 1. Caregiver F stated Resident 1 has been known to have sexual behaviors and staff are to closely monitor Resident 1 and has door alarms on bedroom door and bathroom door. Caregiver F stated ISP's are incorrect for all of the residents and not updated. Specifically, Resident 1's ISP "It's not correct." Caregiver F elaborated that Resident 1 "needs more supervision, like 1:1." Caregiver F stated the facility received multiple emails from the adult day center requesting Resident 1 not to go there due to bullying/harassing consumers at the center. Caregiver F stated Resident 1 had also indicated s/he had sex with another consumer at the center resulting in a pregnancy. Caregiver F also noted incidents when Resident 1 called friends who attend the day center multiple times in a night, resulting in the friends' home blocking Resident 1's number. Caregiver F stated this was about 2 months ago. Caregiver F stated	M 424		

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M 424	Continued From page 29 since then Resident 1's phone use is monitored and Resident 1 can only make phone calls between the hours of 6:00pm-8:00pm in the presence of staff. Caregiver F stated the ISP for Resident 1 is not an accurate reflection of Resident 1. On 07/23/2024 at 11:00AM, Surveyor interviewed Caregiver D. Caregiver D stated s/he has been at the home for about 5 years and works the 2:00pm-10:00pm shift. Caregiver D indicated s/he was familiar with Resident 1. Caregiver D acknowledged Resident 1 has sexual behaviors towards other residents and has had an increase in sexual expressions recently. Caregiver D stated there are alarms on Resident 1's doors now so they know when Resident 1 leaves his/her room. Additionally, Caregiver D stated Resident 1 can only make phone calls between the hours of 6:00pm-8:00pm and the call is to be supervised by a staff member. Caregiver D was unsure when the phone limits started but said it was due to Resident 1 calling consumers from adult day center trying to convince them to be his/her boy/girlfriend. Per Caregiver D, Resident 1 called his/her boy/girlfriend 42 times in an hour resulting in the boy/girlfriend's home blocking Resident 1's number. Caregiver D was unaware if any of this was documented in Resident 1's record or ISP. On 07/24/2024 at 1:15PM, Surveyor interviewed Guardian G. Guardian G verified s/he is the guardian for Resident 1. Guardian G stated s/he was unaware of any phone issues. Guardian G was aware of Resident 1 walking over to friends house but unaware of the multiple phone calls to the residence. Guardian G stated Resident 1 should be able to pickup the phone and use it when s/he wants. Guardian G stated s/he was never told Resident 1 could only use the phone	M 424		

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M 424	Continued From page 30 between 6:00pm-8:00pm. On 07/29/2024, Surveyor emailed Program Director B requesting all client right limitations for Resident 1. Program Director B replied to the email, "There was not." On 07/31/2024 at approximately 10:00AM, Surveyor interviewed Program Director B. Program Director B indicated s/he was responsible for updating ISP's when changes occurred. Program Director B thought Resident 1's guardian had consented to the phone restriction from 6-8pm at night but Resident 1 could receive calls other times of the day. Program Director B stated s/he had met with staff monthly regarding Resident 1's behaviors and they had discussed interventions such as monitoring more frequently, making sure Resident 1 wasn't alone in living room or sunroom with another resident, and door alarms on bedroom door and bathroom door. Program Director B acknowledged Resident 1's current ISP did not reflect the phone usage restrictions nor the sexual behaviors towards other residents and interventions to be used to prevent sexual behaviors toward other residents. On 08/05/2024 at 9:00AM, Surveyor interviewed Licensee A. Licensee A acknowledge the ISP's did not contain the sexual behaviors towards residents and interventions to prevent further behaviors. Additionally, Licensee A stated Program Director B and Licensee A looked for documentation regarding guardian's consent for restricting the phone usage and were not able to locate anything. Cross reference: M 0230 DHS 88.04(2)(f) Condition of Risk	M 424		

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M 450	88.07(2)(b)6 NOTIFICATION OF CHANGES Notifying the placing agency, if any, the guardian, if any, of any significant changes in the resident's medical condition, including any life-threatening, disabling or serious illness, any illness lasting more than 3 days, an injury sustained by the resident, medical treatment needed by the resident or the resident's absence from the home for more than 24 hours. This Rule is not met as evidenced by: Based on record review and interview, the service provider did not notify the placing agency or the guardian of a significant increase in sexual behaviors for 1 of 4 (Resident 1) residents reviewed. Resident 1 was noted to have sexual behaviors in May 2023 with an increase in frequency thereafter. The MCO (Managed Care Organization) and Resident 1's guardian were not made aware of the increase in behaviors until July 2024. Findings Include: On 07/11/2024, the department received a complaint regarding resident behaviors. On 07/22/2024, Surveyor conducted an onsite visit to the home. A sample of residents was selected including the record of Resident 1.	M 450			

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M 450	Continued From page 32 Resident 1 was admitted to the facility on 10/03/2017 with diagnoses to include autistic disorder, anxiety disorder, cognitive impairment, intermittent explosive disorder, mood disorder and articulation disorder. Resident 1's most recent ISP (Individualized Service Plan) dated 06/17/2024 stated Resident 1 requires 24 hour supervision with 30 minutes unsupervised allowed. Resident 1's ISP indicates Resident 1 is "at risk to abuse other vulnerable adults...sexually inappropriate behavior. Responds to redirection: - Note: [Resident 1] seems to crave physical contact and will often seek this out from staff. It is ok to give [him/her] a hand shake or verbal praise, but staff needs to be very careful because [Resident 1] could misinterpret what any physical contact means. If [Resident 1] hugs, invades personal space, or touches anyone without permission staff must redirect [him/her] and ask for personal space, as well as let [him/her] know that is not appropriate." Resident 1 was discharged from facility on 07/24/2024. On 07/23/2024 through 07/29/2024, Surveyor reviewed Resident 1's incident reports and resident daily notes. The following was noted: *A resident profile note in the Rtask system titled "Hiding/masturbation Behavior" dated 05/24/2023 stated, "At dinner time, [Resident 1] did not reply to staff calling for [him/her] including calling down to the basement. Staff then began looking for [him/her] inside and outside the house and then went down to the basement while still asking out if [s/he] was there. Upon turning the lights on, [Resident 1] sat up on one of the couches fully nude. Staff asked if [s/he] was ok, and why didn't [s/he] reply. [Resident 1] replied that [s/he] did and was just "working out."	M 450		

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M 450	Continued From page 33 *A resident profile note dated 9/9/23 stated, "I noticed [Resident 1] laying with [his/her] head on [Resident 2's] lap. Since [Resident 2] does not have the ability to verbalize what [s/he] is feeling or experiencing it is imperative that we document this behavior or any other physical contact beyond expected friendship contact is seen." *A resident profile note dated 9/24/23 stated, "Incident follow-up Upon checking the tablet for sites that we wouldn't want [his/her] housemate to visit, this writer discovered two visits to pornhub.com and xnxx.com dated September 21, 2023 beginning at 8:56 PM." *A resident profile note dated 2/19/24 indicated Resident 1 was making statements about having sex with another day center consumer. A statement from the note stated, "We are at this time unsure if this is true, but this is concerning and needs to be known by all parties involved." *A resident profile note dated 4/29/24 indicated day center called the house to speak with program manager about Resident 1's recent increase over the weekend of phone harassments of 3 fe/males that s/he attends day center with. *A resident profile note dated 5/21/24 stated, "The consumer was discovered going into another consumers bedroom on the opposite side of the house in the middle of the night. It has been a recent occurrence that this consumer will avoid detection by going through [his/her] shared bathroom and housemates bedroom, thereby avoiding [his/her] alarm from going off so as to remain undetected. It is imperative that all staff keep this consumer under constant watch for the	M 450		

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M 450	Continued From page 34 safety of the consumer as well as the other consumers in the house." *On 07/05/2024 an incident report stated, "Staff just finished cleaning dinner around 5:30pm and entered the living room to find [Resident 1] sitting next to [his/her] housemate on the couch. At this time [Resident 1] was holding housemates hands in between [his/her] own legs near [his/her] genitals (over [his/her] pants). Staff quickly redirected the housemate to another location of the house." *On 07/18/2024 an incident report stated, " On 7/18/2024 around 7pm, this staff went downstairs to question [Resident 1] if [s/he] had taken another consumer's tablet. This staff asked [Resident 1] to come out of the room [s/he] was in, but [s/he] couldn't open the door and was locked inside. This staff attempted multiple times until after 8pm to get [Resident 1] to open the door, but found [s/he] was legitimately locked inside the room. After working together to attempt to open the door, this staff was unsuccessful. This house's PD[Program Director], [Resident 1] admitted to this staff that [s/he] had taken the other consumer's tablet. This staff then went onto the tablet's search history and found pornography searches with timestamps from within the hour [Resident 1] was in that locked room." On 07/23/2024 at 10:20AM, Surveyor interviewed Provider Engagement Professional H. Provider Engagement Professional H verified s/he worked for the Managed Care Organization (MCO) that is funding the placement of Resident 1. Provider Engagement Professional H stated Resident 1 does have a BSP (behavior support plan) and referenced page 9 of the ISP which references Resident 1's desire to have physical contact and	M 450			

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M 450	Continued From page 35 will seek that out from caregivers as noted above. Provider Engagement Professional H stated Resident 1 is to be redirected with sexual expressions. Provider Engagement Professional H stated the first incident they were made aware of was the 07/05/2024 incident, however, the MCO did not get notified about the incident until 07/11/2024. Provider Engagement Professional H stated per the contract with the MCO, the provider should have notified the MCO within 24 hours. Provider Engagement Professional H stated they were unaware of any other incidents or allegations until they started to look into the 07/05/2024 incident. Provider Engagement Professional H indicated case managers are to go to the home every month and nothing was every shared with them about these types of behaviors. Provider Engagement Professional H stated it was a "poor job" of communication. Provider Engagement Professional H indicated Resident 1's case managers were looking at moving Resident 1 as soon as possible. On 07/24/2024 at 1:15PM, Surveyor interviewed Guardian G. Guardian G verified s/he was the guardian for Resident 1 for the last 1 1/2 years. Guardian G had indicated Resident 1 moved this morning to a new home. Guardian G stated s/he got an email last week from the facility regarding the incident on 07/05/2024, however, no one had mentioned anything happening like this before. Guardian G stated staff would say Resident 1 gets too close and staff would redirect Resident 1 and things would be fine. Guardian G stated staff never indicated anything sexual going on until the 07/05/2024 incident. On 08/05/2024 at 10:00AM, Surveyor interviewed Licensee A. Licensee A stated s/he would look in Resident 1's records to see if there was any	M 450		

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M 450	Continued From page 36 documentation regarding notification to the MCO or Guardian G regarding the sexual behaviors. At the time of writing, Surveyor received no additional documentation indicating Guardian G or the MCO staff were made aware of the increase in sexual behaviors until 7/11/2024, 6 days after the incident occurred. Cross reference: M 0230 DHS 88.04(2)(f) Condition of Risk	M 450			